

HOUSE No. 2229

The Commonwealth of Massachusetts

PRESENTED BY:

David M. Rogers and Greg Schwartz

To the Honorable Senate and House of Representatives of the Commonwealth of Massachusetts in General Court assembled:

The undersigned legislators and/or citizens respectfully petition for the adoption of the accompanying bill:

An Act to expand equity and access to patient centered care for substance abuse disorder.

PETITION OF:

NAME:	DISTRICT/ADDRESS:	DATE ADDED:
<i>David M. Rogers</i>	<i>24th Middlesex</i>	<i>1/16/2025</i>
<i>Greg Schwartz</i>	<i>12th Middlesex</i>	<i>1/16/2025</i>

HOUSE No. 2229

By Representatives Rogers of Cambridge and Schwartz of Newton, a petition (accompanied by bill, House, No. 2229) of David M. Rogers and Greg Schwartz for legislation to expand equity and access to patient centered care for substance abuse disorders. Mental Health, Substance Use and Recovery.

[SIMILAR MATTER FILED IN PREVIOUS SESSION
SEE HOUSE, NO. 2011 OF 2023-2024.]

The Commonwealth of Massachusetts

In the One Hundred and Ninety-Fourth General Court
(2025-2026)

An Act to expand equity and access to patient centered care for substance abuse disorder.

Be it enacted by the Senate and House of Representatives in General Court assembled, and by the authority of the same, as follows:

1 SECTION 1: Notwithstanding any special or general law to the contrary, the Department
2 of Public Health and the Massachusetts Board of Registration in Medicine shall develop, or
3 provide for, a healthcare provider education campaign that encourages the adoption of all FDA-
4 approved medications for the treatment of alcohol and opioid use disorders. The goal of the
5 campaign is to increase the number of providers offering all FDA-approved medications, directly
6 or by referral, along with counseling and other appropriate support services. The training shall
7 include, but not be limited to guidelines and best practices for:

8 A. Screening and Assessment;

9 B. Toxicology screens;

C. Detoxification/withdrawal management and induction to relapse prevention medication

D. Treatment Plans, including counseling frequency and type, and an informed consent process to guide medication and treatment decisions and selection

E. Addressing Co-occurring mental health disorders

F. Reducing disparities in health outcomes for underserved communities experiencing substance use disorder

G. Care Coordination;

H. Appropriate Length of Treatment; and

I. Relapse Prevention

The training developed or provided shall be accepted by the board as up to 2 continuing professional development credits.

SECTION 2: Notwithstanding any special or general law to the contrary, the Department of Public Health and the Bureau of Substance Abuse Services shall establish a peer mentoring program that supplements the healthcare provider educational campaign by providing a network for peer-to-peer trainings, materials, and prescriber and clinical team support. Peer mentors should have strong credentials, expertise and clinical experience with all FDA-approved medications for the treatment of substance use disorder. Mentors shall provide coaching for providers licensed or certified by the Department of Public Health. The Department shall prioritize the efforts of the peer mentor program for providers serving geographic areas and racially and ethnically diverse populations of the Commonwealth identified by the Department

where access to medication assisted treatment is limited. Provided further, that said training program shall include, but not be limited to the following criteria: patient eligibility, optimal selection criteria, placement matching, patient engagement, team coaching and coordination, withdrawal management and induction, dosing and administration, clinical evaluation and laboratory monitoring, side effect management, co-occurring disorders management, drug-drug interactions, treatment retention, managed care interactions, and termination of medication.

SECTION 3: There shall be a grant program established to support providers who demonstrate the ability to offer all FDA-approved medications for substance use disorders, along with counseling and other supports, directly or by referral. Providers will be eligible to apply for funding to add a staff person(s) to support the expanded services. In addition to, or in substitution of state funding, the grant program may utilize applicable federal grants and state trust funds.

SECTION 4: The department of public health shall create an inventory of health care providers treating patients with medications to measure adoption of offering all FDA-approved treatment options across the Commonwealth. They shall also submit a report to the house and senate committees on ways and means and the joint committee on mental health and substance use, and recovery on the number of providers trained and any identified obstacles to expanding the number of providers offering all FDA-approved medications by January 1, 2020.